Suspected Adverse Drug Reaction Reporting Form

Report type	Initial
AMC report number	Unknown
Worldwide unique number	Unknown

A Patient Information

Patient initials	Mr.R.K.
Case ID	ADR-2026-014
Age / date of birth	58 years
Sex	Male
Weight	72 kg
Height	5.8 in
Care setting	Outpatient, then admitted to General Medicine ward
Relevant medical history	Type 2 diabetes (8 years); hypertension (5 years); dyslipidemia; gout (diagnosed 3 weeks before starting therapy)
Known allergies	None
Liver / kidney disease	None; baseline creatinine 1.0 mg/dL
Habits	Non-smoker; occasional alcohol

B Suspected Adverse Reaction

Date of onset	Day 21 (itching); Day 25 (fever, rash, facial swelling)
Date of recovery	About week 6 after treatment began (LFTs and creatinine normalized)
Reaction / event description	Generalized itchy maculopapular rash starting on the trunk and spreading to the limbs, with fever (101.4 F), facial edema, tender cervical and axillary lymphadenopathy, and mild hepatomegaly. Labs showed eosinophilia, hepatitis and acute kidney injury. No mucosal erosions, blisters or Nikolsky sign.
Diagnosis	DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms)

Clinical details

Reaction evidence, seriousness and suspected medication

B Key Laboratory Evidence

Test	Result	Reference
Eosinophils	1,400/uL (14%)	<500
WBC	13,400/uL	4,000-11,000
ALT / AST	210 / 165 U/L	<40
Bilirubin	1.4 mg/dL	<1.2
Serum creatinine	1.9 mg/dL	Baseline 1.0
Viral hepatitis markers	Negative	
Blood culture	No growth	

B Seriousness and Outcome

Seriousness criteria	[ ] Death [ ] Life-threatening [x] Hospitalization (initial or prolonged) [ ] Disability/incapacity [ ] Congenital anomaly [x] Other medically important condition: multi-organ involvement (liver, kidney)
Seriousness / severity	Serious / Severe
Outcome	[x] Recovered (with treatment) [ ] Recovering [ ] Not recovered

	[ ] Fatal [ ] Unknown

C Suspected Medication

Suspected drug (generic)	Allopurinol
Brand / manufacturer	Unknown
Batch / lot and expiry	Unknown
Dose	300 mg (full dose, no titration)
Route / frequency	Oral, once daily
Indication	Chronic gout (urate lowering)
Date started	Day 0
Date stopped	Day 28 (on admission)
Duration before reaction	About 21 to 25 days
Action taken	[x] Drug withdrawn [ ] Dose reduced [ ] Dose increased [ ] Not changed [ ] Unknown
Dechallenge	Positive: reaction improved after withdrawal (with corticosteroids)
Rechallenge	Not done (contraindicated)

Treatment and assessment

Concomitant medicines, management and causality review

D Concomitant Medications

Include prescription medicines, self-medication and over-the-counter products.

Drug	Dose	Indication	Dates	Suspected?
Colchicine	0.5 mg BD	Gout flare prophylaxis	Day 0 to Day 14	No; stopped 7 days before onset
Metformin	500 mg BD	Diabetes	6 years, ongoing	No
Amlodipine	5 mg OD	Hypertension	4 years, ongoing	No
Atorvastatin	20 mg OD	Dyslipidemia	2 years, ongoing	No
Paracetamol	500 mg PRN	Fever/pain	From Day 24	No; started after rash began

E Management of the Reaction

Drug withdrawal	Allopurinol stopped immediately
Supportive care	IV fluids, topical emollients, oral antihistamines
Specific treatment	Prednisolone 1 mg/kg/day (about 72 mg/day), tapered
Response	Gradual improvement over 10 days; LFTs and creatinine normal by week 6

Reporter and submission

Final assessment and reporter details

F Other Drugs Considered and Excluded

Assessment	Colchicine stopped before onset and is not typical for DRESS; metformin, amlodipine and atorvastatin had been used for years without change; paracetamol was started after onset.

G Reporter Details

Reporter name	Mansi Rayau
Designation	[ ] Physician [x] Pharmacist [ ] Nurse [ ] Other
Institution / address	To be filled

Phone / email	To be filled
Date of report	To be filled
Signature	_____

Confidentiality
Patient identity and reporter details are kept confidential. Submit this report to the nearest PvPI Adverse Drug Reaction Monitoring Centre (AMC), or via the ADR PvPI app or helpline 1800-180-3024.